INTEGRATING INTERSEX PERSPECTIVE INTO HEALTHCARE POLICIES FOR INCLUSIVE PRACTICES IN NIGERIA

(A Policy & Practice Toolkit for Healthcare Providers and Policymakers)

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intersex
NIGERIA
CENTER FOR HEALTHCARE DEVELOPMENT & YOUTH EMPOWERMENT

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I am deeply grateful for the opportunity to participate in the 2023 UN Rights and Religion Fellowship by Outright International. The fellowship strengthened my advocacy and knowledge-creation work and provided a powerful platform to amplify the rights and inclusion of intersex people in Nigeria and around the world.

Through the fellowship, I collaborated with intersex individuals in Nigeria to develop a comprehensive toolkit; Integrating Intersex Perspectives into Healthcare Policies for Inclusive Practice in Nigeria to help embed intersex perspectives across health policy and practice nationwide. This toolkit offers practical guidance for healthcare providers on respectful, appropriate care; equips government institutions and policymakers to understand the unique challenges intersex people face; and underscores the need for legislation that protects intersex rights and dignity.

I extend my appreciation to Outright International for its steadfast support and global leadership, and to every intersex individual and ally who contributed to this work. My special thanks go to Kimberly Zieselman for her thoughtful review and editorial guidance. I am honored to have authored a resource I believe will serve healthcare professionals, government institutions, and civil society organizations in building a more inclusive healthcare system in Nigeria.
Together, let us continue advancing the rights and dignity of intersex people working toward a future in which everyone, regardless of intersex status, is treated with fairness, respect, and equality in Nigeria.

With deep appreciation,

Obioma Chukwuike
Executive Director
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SECTION 1 — About This Toolkit

Introduction

Intersex persons are individuals who are born with innate variations in their chromosomes, hormones, or reproductive anatomy which manifest as variations in sex characteristics that do not fit typical binary notions of male or female bodies.

To create an inclusive society that embraces diversity, we must address the unique needs and challenges faced by intersex people in Nigeria's healthcare system.

Purpose

The purpose of this toolkit, targeted at healthcare providers, government institutions, policy makers, and civil society organizations (CSOs), is to raise awareness and provide guidance on integrating intersex perspectives into healthcare policies for inclusive practices. It aims to address the specific needs and experiences of intersex individuals in the Nigerian healthcare system, ensuring that their rights to dignity, bodily autonomy, and non-discrimination are respected and protected.

Through this toolkit, healthcare providers will have more insight on guidance and knowledge to provide appropriate and inclusive care to intersex patients. It will outline best practices in diagnosis and support, while considering the specific health concerns and challenges faced by intersex individuals. Additionally, the toolkit will highlight the importance of informed consent and provide guidelines on involving intersex individuals in decision-making processes regarding their own healthcare.



Government institutions and policy makers play a crucial role in developing and implementing healthcare policies that address the needs of all citizens, including intersex individuals. This toolkit will serve as a resource to inform policy development and reform, ensuring that intersex perspectives are incorporated into healthcare policies to promote inclusivity and safeguard the rights of intersex people.

CSOs advocating for the rights and well-being of intersex individuals can utilize this toolkit to augment their efforts. It will provide them with evidence-based information and practical recommendations to lobby for policy changes and raise awareness on intersex issues. By collaborating with healthcare providers, government institutions, and policy makers, CSOs can work towards creating a healthcare system that is sensitive, respectful, and inclusive of intersex people's unique needs in Nigeria.

Target Audience

1. Government agencies and institutions, who play a crucial role in initiating and implementing policies that will promote intersex-affirming inclusion in Nigeria.

2. CSOs, who are very important advocates for rights of marginalized and vulnerable groups, including intersex individuals, and greatly contribute to shaping public opinions and empowering intersex individuals.

3. Medical practitioners, who are frontline healthcare providers and have the responsibility to ensure that intersex individuals feel seen, heard, and supported in their journey towards achieving optimum health and wellbeing support, needs and services.



How to Use This Toolkit

This toolkit is designed as a practical resource to support the integration of intersex perspectives into healthcare policies and practices in Nigeria. It brings together human rights standards, medical ethics, policy guidance, and lived experiences to help different stakeholders strengthen inclusive healthcare systems. Readers are encouraged to engage with the sections most relevant to their roles, while recognising that sustainable change requires collaboration across sectors.

Who Should Use This Toolkit

Government institutions and policymakers can use this toolkit to inform policy development, legal reform, and the design of inclusive healthcare systems that respect bodily autonomy and human rights. Sections on legal frameworks, policy design, stakeholder engagement, and recommendations for government agencies provide guidance for integrating intersex inclusion into national health strategies.

Healthcare providers and medical educators can use this toolkit to improve their understanding of intersex realities and strengthen ethical, respectful, and affirming care. The sections on medical ethics, informed consent, mental health, inclusive healthcare services, and the guidelines for healthcare providers offer practical steps for delivering patient-centred care.

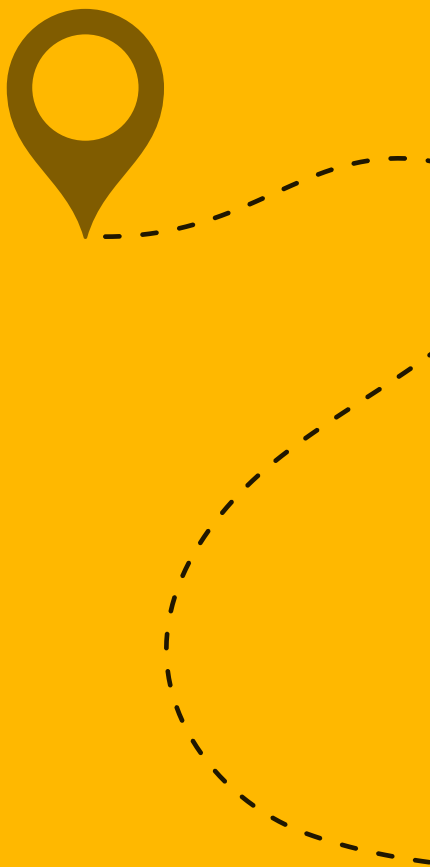
Civil society organisations (CSOs) and advocates can use this toolkit as an advocacy and awareness-raising resource. It supports organisations to strengthen community engagement, develop training initiatives, and collaborate with policymakers and healthcare institutions to advance intersex inclusion.

How to Navigate the Toolkit

This toolkit is structured to move from foundational knowledge to practical action:

- **Understanding Intersex Realities:** Provides essential concepts, myths and facts, and an overview of human rights issues affecting intersex people.
- **Legal and Policy Context:** Outlines international human rights standards and the Nigerian legal landscape relevant to intersex inclusion.
- **Developing Inclusive Policies:** Offers principles, stakeholder engagement strategies, and practical steps for designing and implementing intersex-inclusive healthcare policies.
- **Practical Guidance and Recommendations:** Includes actionable guidelines for healthcare providers, recommendations for government agencies, and calls to action for CSOs.

Readers may choose to follow the toolkit sequentially or focus on specific sections depending on their needs and responsibilities.



Using the Toolkit for Training and Implementation

This toolkit can be used in multiple ways:

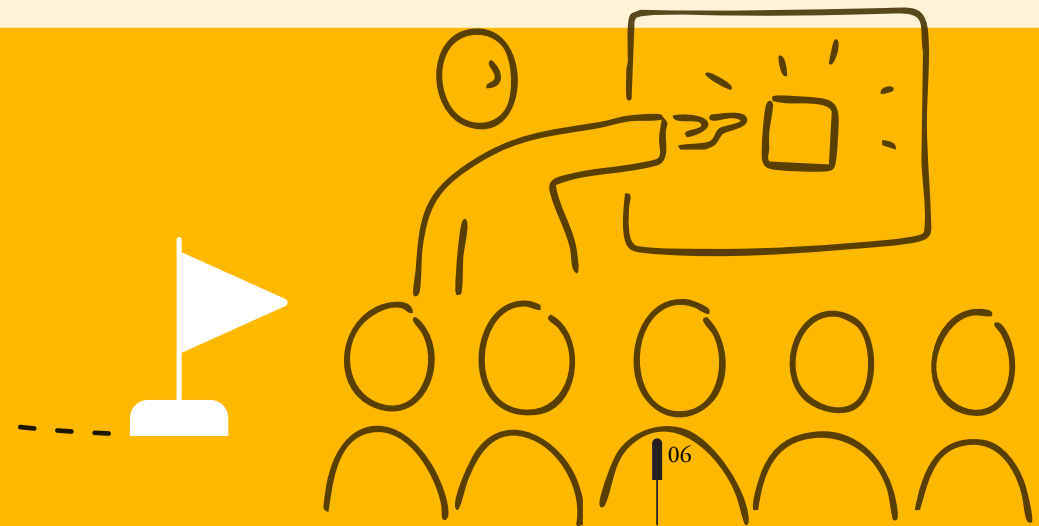
- As a **training resource** for healthcare professionals, policymakers, and advocates.
- As a **policy reference** for developing inclusive guidelines, protocols, and legislation.
- As an **advocacy tool** to promote awareness, collaboration, and systemic change.
- As a **framework** for dialogue between intersex communities, healthcare institutions, and government agencies.

Principles for Applying This Toolkit

When using this toolkit, readers are encouraged to:

- Centre the voices and lived experiences of intersex individuals.
- Uphold human rights principles, including dignity, bodily autonomy, and non-discrimination.
- Promote meaningful participation of intersex communities in decision-making processes.
- Adapt the recommendations to local contexts while maintaining international human rights standards.

By applying the knowledge and guidance within this toolkit, stakeholders can contribute to building a healthcare system in Nigeria that is inclusive, respectful, and responsive to the diverse realities of intersex people.



Understanding Intersex

Intersex is a term used to describe individuals born with innate reproductive or sexual anatomy that does not conform to what is typically considered a "female" or "male" body. In fact, intersex is a completely natural bodily variation in humans that, in some cases, may not even be visible at birth but appear during puberty or even into adulthood. A person might learn that they have an intersex body at a very early age or later in life. Some intersex people never find out at all.

The term “intersex” is an umbrella term for the spectrum of variations in sex characteristics that naturally occur within human biology. The term acknowledges the fact that sex is a spectrum and that people with variations in sex characteristics other than male or female exist.

Most human sex characteristics exist at birth. However, intersex people often face discrimination or violence simply due to their innate sex characteristics. The fact that someone has an intersex body can become obvious at various points in their lives: at birth, during childhood, puberty, or even adulthood. Depending on the specific life circumstances and the level of taboo existing in their community, their natural physical intersex traits put them at risk of harm due to stigma. Intersex is considered taboo in many parts of the world, particularly in Africa. According to the United Nations, at least 1.7% of the population, or, globally speaking, as of 2019, 131 million people have been born with intersex traits. This means that nearly one in every 60 people has a variation in sex characteristics and does not fit the typical medical and societal definition of male or female, putting them at risk of discrimination and other human rights violations at any age.[1]

There is a wide variety of differences among intersex variations, including differences in genitalia, chromosomes, gonads, internal reproductive organs, hormone production and response, and/or secondary sex traits (such as body hair).

According to studies published in the Netherlands in 2014, at least one in every 200 people are at risk of being subjected to invasive surgeries and other medical interventions, such as hormonal treatment, if medical professionals diagnose them with a "Disorder of Sex Development" (DSD) or an "unspecified" diagnosis, such as "unspecified malformation of the male/female genitalia.”

Many of these interventions, particularly surgeries, are irreversible, deferrable, non-emergent interventions on healthy bodies, and they are frequently performed during infancy and childhood without personal or fully informed consent.[2]

[1] United Nations Office of the High Commissioner for Human Rights. (2019). Background note on human rights violations against intersex people. https://www.ohchr.org/en/documents/thematic-reports/background-note-human-rights-violations-against-intersex-people

[2] Carpenter, M. (2016). The human rights of intersex people: Addressing harmful practices and rhetoric of change. Reproductive Health Matters, 24(47), 74–84. https://doi.org/10.1016/j.rhm.2016.04.003

"Disorder of Sex Development (DSD)" *is a medical umbrella term, which was introduced in 2006 by a Clinician Consensus Statement.*[3] *Together with new categories of "syndromes", it replaced the older medical terms. Some clinicians use DSD to stand for "differences of" or "diverse" sex development.* ***However, in all its forms the term pathologises healthy variations of sex characteristics and refers to intersex sex characteristics as characteristics that are "deviant" from the norm of male and female bodies and thus need to be "disambiguated" or "fixed". The term "DSD" does not align with human rights standards, and is only used in this document and the accompanying appendix in direct quotations or when referring to medical concepts that use the term.***

Intersex bodies are often perceived as neither male or female, and many in society want to "fix" otherwise healthy bodies to meet a binary notion of sex and gender. As a result, intersex people are vulnerable to stigma, violence, and harmful practices, such as forced or coercive medical interventions aimed at making their bodies more typically female or male.

Common Intersex Myths, and Facts

Myth: Everybody is born with either a male or female body

Fact: People often assume that the world is divided neatly into two groups of people, male and female, and that everyone's biological and genetic characteristics fit into one of these two categories. But this is not always the case. There are millions of people around the world who have sex characteristics that do not fit typical binary notions of male or female bodies.

Myth: Being intersex is extremely rare.

Fact: The scientific consensus is that between 1.5-2% of people are intersex.[4] It is actually not too uncommon to be intersex, and given that many people might not have easily detected intersex characteristics, the number could be far higher. Some people are not born with obvious intersex characteristics, and instead they might become obvious at puberty or later in life, while some may never find out.

Myth: Intersex is a medical condition or birth defect which needs 'correcting'.

Fact: Since the 1950s, and after the work of disgraced gender scientist John Money had made an impact across Europe and North America, operations to 'correct' intersex people became more popular and were exported globally. Intersex people are rarely harmed by their natural bodily variations, and more often than not, 'treatment' causes lifelong physical and emotional damage. It is by no means necessary to 'correct' an intersex person, and the myth which supports this is one which is particularly damaging to the intersex community.[5]

[3] Hughes IA, Houk C, Ahmed SF, Lee PA; LWPES Consensus Group; ESPE Consensus Group. Consensus statement on management of intersex disorders. Arch Dis Child. 2006 Jul;91(7):554-63. doi: 10.1136/adc.2006.098319. Epub 2006 Apr 19. PMID: 16624884; PMCID: PMC2082839.

[4] https://ihra.org.au/16601/intersex-numbers/

[5] Amnesty International. (2017). First, do no harm: Ensuring the rights of children with variations of sex characteristics in Denmark and Germany (Index No. EUR 01/6086/2017). https://www.amnesty.org/en/documents/eur01/6086/2017/en/

Myth: Intersex people have 'both' genitals, or are 'both' sexes.

Fact: The truth is, it's complicated. Sex is a series of traits and spectrum and there are many ways human bodies can develop. Having both a penis and vagina is not one of them. Particularly the media tends to publicize intersex people as “born with both organs” which is false and only perpetuates harmful stigma and stereotypes.

Myth: All intersex people all have ambiguous genitalia at birth and have had surgeries.

Fact: Not all intersex people have ambiguous genitalia at birth, not all intersex experiences involve surgery, and not all intersex medical harm is surgical, other harmful medical practices, Non-consensual or coerced hormones to “normalize” levels or induce/stop puberty without full, age-appropriate information or alternatives. Forced or pressured vaginal dilation in youth. Non-consensual medical photography. Withholding or delaying medically necessary care due to bias or confusion. Prenatal drug exposures intended to “prevent” intersex traits e.g., steroids for Congenital Adrenal Hyperplasia (CAH) without informed consent. Sharing diagnoses with schools, employers, or relatives without permission.

Myth: Intersex people are transgender.

Fact: The word “transgender” or trans is an umbrella term for people whose gender identity is different from the sex they were assigned at birth. The word “intersex” relates to innate physical variations in sex characteristics, and not to an internal sense of identity. It is therefore untrue that all (or even most) intersex people are trans.

Myth: Intersex is a taboo or a bad omen

Fact: Intersex is a biological and innate human diversity in sex characteristics and it is not a taboo. The cultural misperception that intersex people are somehow evil or bring bad luck is an extremely harmful misunderstanding that results in painful stigma and harm for both the intersex individual and their family.

Myth: Intersex people can't have sex, get pregnant, or have children.

Fact: Many intersex variations include mostly typical internal reproductive anatomy. Everyone is different. There are intersex people who can have periods and carry children, and intersex people who can produce sperm. There are some who can't carry children or produce sperm.



Intersex and Gender

Intersex is a sex characteristic, just like being female or male, it is not a gender identity. Intersex and gender identity are complex and interesting topics that intersect in so many ways. These intersections stem from the fact that intersex individuals face a multitude of challenges in understanding and navigating their own gender identity. Because of the stigma attached to intersex people and their bodies, many may identify with the assigned sex at birth based on the predominant internal and physical sex characteristics at birth, for intersex persons biological assigned sex is not the sole determinant of their gender identity; a multitude of factors as their bodies mature might change their internal and/or physical sex characteristics appearance from the sex assigned at birth to an opposite sex.

Intersex people come from all socioeconomic backgrounds, culture, race, ethnicities, faiths, and political ideologies. Intersex is not itself a sexual orientation or even a gender identity. Though some individuals with intersex traits identify with "intersex" as their sex the majority don't, and most identify as either male or female. Intersex individuals may also identify with a non-binary gender identity. Intersex is not the same as transgender. However, like trans people, some intersex individuals grow up to identify with a different gender than the gender they were given at birth, most times due to changes that occur in their body anatomy during sex characteristic development.

Most societies are structured along the supposed binary of sexes. These societal systems make those who do not fit into the male-female sex binary particularly vulnerable to violations of bodily integrity, discrimination, harassment, violence in medical settings, and bullying at school and on the job. The human rights violations that intersex people face is rooted in what the Bioethicist, George Annas, has called "monster ethics," as pointed out in a surgery-critical article published in the Journal of Pediatric Urology in 2016.[6] *"Babies with atypical sex are not [considered] yet fully human, and so not entitled to human rights. Surgeons make them human by making them recognizably male or female, and only then may they be regarded as entitled to the sexual and medical rights and protections guaranteed to everyone else by current ethical guidelines and laws."*[7]



[6] Journal of Pediatric Urology, ISSN: 1477-5131, Vol: 12, Issue: 6, Page: 436-437, 2016.
[7] Ehrenreich, N., & Barr, M. (2016). Intersex surgery, female genital cutting, and the selective condemnation of "cultural" practices. Journal of Pediatric Urology, 12(6), 404–409. https://doi.org/10.1016/j.jpurol.2016.08.013

Overview of Key Intersex Issues

Human Rights: Intersex Inclusion and Protection from Harmful Practices

Human rights are universal and indivisible, which means they apply to everyone, including intersex people. The Universal Declaration of Human Rights (UDHR) states that every human being, regardless of race, color, gender, language, religion, political or other opinion, national or social origin, property, birthplace, or other status, is born free and equal in dignity and rights.[8] Despite the fact that no specific provision currently refers to intersex people, the rights contained in international human rights treaties apply to all people, including intersex people, through the conventions' open-ended non-discrimination clauses.

People always immediately ask, "Is it a boy or a girl?" when they welcome a new baby. At first glance, the seemingly innocent question reveals the deeply ingrained nature of sex and gender roles in our society, along with the binary classification of sexes. Also, the fact that there is no natural comparison to the rigid line we use to split men and women into two groups that can't mix, demonstrates how little we understand about sex and its classifications.

After birth, as children, adolescents, and adults, intersex people face common violations of their physical integrity, including medical interventions without personal, prior, persistent, and fully informed consent. One of the biggest issues facing the intersex community is the non-consensual surgeries performed on intersex children. These are non-lifesaving procedures intended to change genital appearance and reproductive anatomy to fit arbitrary norms. Nonconsensual and medically unnecessary practices, such as genital mutilation and infanticide, constantly subject intersex children to violations of their rights to fully informed consent, bodily autonomy, and self-determination. In most cases, this causes psychological trauma as well as severe physical impairments, ranging from sterility, painful scar tissue or a lack of sensation to osteoporosis and urethral issues.

Intersex children face the risk of a disturbed family life due to cultural taboo and medicalization. They are also at a high risk of dropping out of school due to the effects of these medical treatments and bullying over their bodily differences.[9] Intersex adults usually have difficulty finding a job due to a lack of education, physical impairments,

[8] United Nations. (1948). Universal Declaration of Human Rights. (United Nations, 1948, Arts. 1–2)
[9] SIPD Uganda, "Baseline survey on intersex realities in East Africa" (2017).

or a lack of self-esteem caused by social stigma. Intersex people are reported to be living at a higher risk of poverty due to a lack of education as a result of pathologisation and related trauma. At every age in Nigeria, intersex people face stigma, structural and verbal discrimination, harassment, a lack of adequate medical care, a lack of access to needed medication, a lack of legal recognition, and the invisibility of their bodies.

Intersex people are vulnerable to discrimination and abuse in all spheres of life. Their invisibility, exclusion, and general lack of knowledge about intersex issues in society fuel the perpetration of discrimination, stigmatisation, and pathologization with impunity, especially in the absence of intersex-specific non-discriminative laws and policies. In Nigeria, being intersex is considered a taboo, a bad omen, and a generational curse from God as a form of punishment. As a result, individuals who openly identify as intersex face numerous discriminatory practices. Perpetrators usually discriminate on the basis of what they perceive as gender non-conformity on the part of intersex people, in behaviour, appearance, or both.

Some countries are taking steps to end infant intersex surgery and promote intersex rights. For example, Malta, Kenya, Columbia, Spain, South Africa, Iceland, Greece, Germany, Pakistan, one territory in Australia and the Indian state of Tamil Nadu[10] have legislated some provisions to protect intersex people while some have explicitly prohibited intersex surgeries without the individual's informed consent. In 2019, the European Parliament passed a resolution[11] calling on all member states to end surgeries. Portugal[12], France[13], and Uruguay[14] have each passed laws to protect intersex people, but which may still stop short of pausing non-consensual surgeries. In 2018, the High Court of Kenya ordered the government to officially recognize intersex people in official documents, such as birth certificates. This ruling was a significant step toward legal recognition and protection for intersex individuals.



[10] https://www.hrw.org/news/2019/08/29/indian-state-bans-unnecessary-surgery-intersex-children
[11] http://www.europarl.europa.eu/doceo/document/B-8-2019-0101_EN.html?redirect
[12] https://oiieurope.org/portugal-adopts-law-protecting-intersex-people/
[13] https://oiieurope.org/france-lacks-understanding-of-intersex-human-rights-violations/
[14] https://www.impo.com.uy/bases/leyes/19580-2017

Legal and Ethical Frameworks

The international human rights standards for intersex individuals is a critical framework for protecting and promoting the rights and wellbeing of this marginalised minority. Under the international human rights legislation, intersex people's rights are based on concepts such as equality, non-discrimination, physical autonomy, and the right to health.[15,16] The Universal Declaration of Human Rights and subsequent treaties, such as the International Covenant on Civil and Political Rights[17] and the Convention on the Rights of the Child,[18] have provisions that apply to protecting intersex people's rights.

These instruments specifically condemn sex discrimination and other forms of discrimination, promote the right to privacy, and defend the right to physical and mental integrity. Additionally, they acknowledge that every child has a right to the best possible health and emphasises on the importance of informed consent for medical procedures.

Statements, resolutions and recommendations at the United Nations level serve to strengthen the international human rights standard on intersex rights. These texts advocate for the eradication of harmful practices, such as nonconsensual and medically unnecessary surgeries on intersex children, and emphasise the importance of intersex individuals receiving support, healthcare, and inclusive education. While there has been some progress in recognising intersex rights around the world, much more work remains to be done, particularly in Nigeria. Most intersex persons continue to experience stigma, prejudice, and violence, and their voices are frequently silenced. Government institutions, healthcare providers, and civil society organisations must ensure that intersex people's human rights are recognised, protected, and fulfilled in all aspects of life. This includes ending discriminatory practices, ensuring access to proper and quality healthcare, and raising understanding and acceptance of intersex diversity in our society.

Adherence to the standard also requires the abolition of legal gender assignment without consent, the promotion of self-determination, and the recognition of diverse sex characteristics. It emphasises the necessity of intersex persons participating in decision making processes that affect their lives, allowing them to have a voice in policy and practice.

[15]OHCHR's Born Free and Equal (2016) explicitly calls intersex persons a "protected group" under existing instruments.
[16]ICCPR Art. 7 (freedom from torture); CRC Art. 24 (health) and Art. 3 (best interests); Convention against Torture Art. 1–2; OHCHR (2016), pp. 30–31.
[17]International Covenant on Civil and Political Rights, Dec. 19, 1966, S. Exec. Doc. No. 95-2, 999 U.N.T.S. 171.
[18]United Nations. (1989). Convention on the Rights of the Child, 1577 U.N.T.S. 3.

International Human Rights Conventions, Resolutions and Treaties that Safeguard Intersex Rights

The International human rights law lays down obligations for State Parties. The obligation to respect means that States must refrain from interfering with or curtailing the enjoyment of human rights. The obligation to protect requires States to protect individuals and groups against human rights abuses. The obligation to fulfil means that States must take positive action to facilitate the enjoyment of basic human rights.

The principles of human rights include that human rights are universal and inalienable because everyone is born with and possesses the same rights and they can not be taken away. They are indivisible and interdependent because all rights are equal in importance and reliant on none can be fully enjoyed without the other. They apply to all equally without discrimination and all people have the right to participate and be included in decisions that affect their lives. They are upheld by the rule of law and duty bearers are accountable to uphold human rights.

1. Universal Declaration of Human Rights (UDHR):

The first international human rights agreement was the Universal Declaration of Human Rights (UDHR), which was adopted by the United Nations General Assembly in 1948. The UDHR commits States to protecting the equality rights of all people:

Article 1 provides that "All human beings are born free and equal in dignity and rights."

Article 2 provides that "Everyone is entitled to all the rights and freedoms set forth in this Declaration, without distinction of any kind, such as race, colour, sex, language, religion, political or other opinion, national or social origin, property, birth or other status", and

Article 7 provides further that "All are equal before the law and are entitled without any discrimination to equal protection of the law. All are entitled to equal protection against any discrimination in violation of this Declaration and against any incitement to such discrimination." Other key rights set out in the UDHR are discussed further below.

The core principles set out in the UDHR, such as universality, interdependence and indivisibility of rights, equality and non-discrimination, and that human rights simultaneously entail both rights and obligations from duty bearers and rights owners, are reflected in numerous international human rights conventions, declarations, and resolutions that followed from the UDHR.[19]

[19] https://www.un.org/en/about-us/universal-declaration-of-human-rights

2. Convention on the Rights of the Child (CRC):

The CRC, ratified by nearly all UN member states, recognizes the rights of intersex children.

Article 3 ensures that the best interests of the child should be a primary consideration in all actions concerning them, including intersex children.

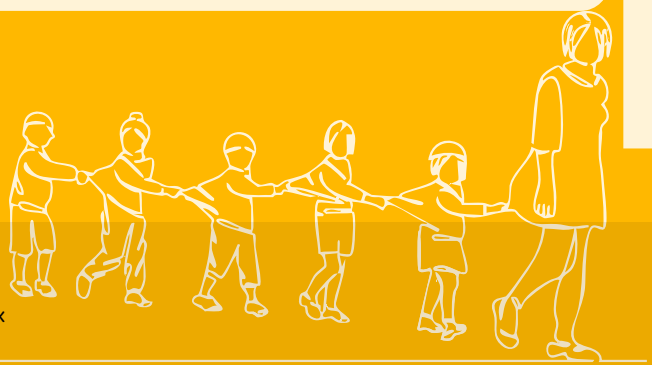
Article 19 protects intersex children from all forms of physical and mental violence, including non-consensual medical interventions, which are sometimes performed without their informed consent.[20]

3. Convention against Torture and Other Cruel, Inhuman or Degrading Treatment or Punishment (CAT):

The CAT, adopted by the UN General Assembly in 1984, prohibits torture and other forms of ill-treatment. It explicitly requires states to prevent and combat torture, including medical interventions without informed consent, that may amount to cruel, inhuman, or degrading treatment. Intersex individuals subjected to non-consensual medical interventions can seek remedies under this convention.[21]

4. Convention on the Elimination of All Forms of Discrimination Against Women (CEDAW):

CEDAW, adopted by the UN General Assembly in 1979, obligates states to eliminate discrimination against women in all areas of life. An intersectional approach has led to the recognition that certain practices, such as unnecessary and non-consensual medical interventions on intersex women, may amount to discrimination and violence against women. CEDAW highlights the importance of ensuring bodily autonomy and integrity for intersex individuals just as it calls for end to female genital mutilation.[22]



[20] https://www.ohchr.org/en/professionalinterest/pages/crc.aspx
[21] https://www.ohchr.org/en/professionalinterest/pages/cat.aspx
[22] https://www.ohchr.org/en/professionalinterest/pages/cedaw.aspx

5. Yogyakarta Principles:

The Yogyakarta Principles, a set of international principles on the application of international human rights law concerning sexual orientation, gender identity, and gender expression, specifically address the rights of intersex people. These principles emphasize the right to bodily autonomy, the prohibition of non-consensual medical interventions, the right to gender recognition, and the right to live free from violence and discrimination.[23]

6. The International Covenant on Economic, Social and Cultural Rights (ICESCR)

The ICESCR, also adopted by the General Assembly in 1966, sets out the economic, social and cultural rights of all persons, to be exercised without discrimination of any kind *"as to race, colour, sex, language, religion, political or other opinion, national or social origin, property, birth or other status"* (article 2(2)). Rights include the right to work (article 6) and to just and favourable conditions of work (article 7), the right to social security (article 9), the right of every person to the highest attainable standard of health (article 12) and the rights of all persons to education (article 13).

Article 2(1) of the ICESCR states each State Party to the Covenant "undertakes to take steps, individually and through international assistance and co-operation, especially economic and technical, to the maximum of its available resources, with a view to achieving progressively the full realization of the rights recognized in the present Covenant by "all appropriate means, including particularly the adoption of legislative measures".[24]

7. African Commission on Human and Peoples' Rights (ACHPR) Resolution 552

Resolution 552, titled "Promotion and Protection of the Rights of Intersex Persons in Africa," was adopted by the African Commission on Human and Peoples' Rights (ACHPR) during its 74th Ordinary Session on March 7, 2023. This landmark resolution calls on African states to end non-consensual and unnecessary genital normalization surgeries on intersex children, recognizing these practices as human rights violations like genital mutilation. The resolution emphasizes the need for comprehensive education programs to protect intersex individuals from discrimination, infanticide, abandonment, and other human rights abuses.[25]

[23] https://yogyakartaprinciples.org
[24] https://www.ohchr.org/en/professionalinterest/pages/cescr.aspx
[25] https://achpr.au.int/en/adopted-resolutions/resolution-promotion-and-protection-rights-intersex-persons

8. UN Intersex Resolution

In 2024, the United Nations Human Rights Council adopted its first-ever resolution specifically aimed at protecting the rights of intersex individuals. This landmark document, titled "Combating discrimination, violence and harmful practices against intersex persons," was led by countries including Australia, Chile, Finland, and South Africa and also notably passed with zero "No" votes.[26] The resolution called for a report and panel to be delivered by the UN OHCHR during the 60th HRC session. The report made a number of groundbreaking recommendations including the protection of intersex people from discrimination and violence in medical settings, schools, employment and more.[27]

Notably, these resolutions passed by both the African Commission on People and Human Rights and the UN Human Rights Council have significantly raised the importance, credibility and awareness of intersex issues and the need for states to protect and include intersex individuals.



[26] https://docs.un.org/en/A/HRC/55/L.9
[27] UN Human Rights Council 60th Session, Discriminatory laws and policies, acts of violence and harmful practices against intersex persons, Report of the Office of the UN High Commissioner for Human Rights, August 8, 2025. https://docs.un.org/en/A/HRC/60/50

The Nigerian Legal Context

In Nigeria, as in many other African countries, intersex persons experience stigma, discrimination, forced medical procedures without adequate and informed consent, as well as a more generalized 'invisibility' in law, regulations and policy, which impacts their access to services and rights to health, welfare and development.

In Nigeria, there are no laws particularly protecting the rights of intersex individuals. Nigeria's current laws are mostly based on the two-sex model which does not take into account the specific needs and experiences of intersex people. For example, the Violence Against Persons Prohibition Act of 2015 (VAPP) aims to eradicate violence in both private and public spheres, outlawing all forms of violence against individuals, especially women and girls, who suffer disproportionately. It provides maximum protection and effective remedies for victims, while the Child Rights Act (CRA) safeguards the rights of every child in Nigeria. Although we could use these laws to advocate for the rights of intersex children and adults, their numerous gaps and nuances could potentially limit our advocacy efforts.[28]

The 1999 Constitution of the Federal Republic of Nigeria states that everyone has the right not to be treated unfairly or discriminated against because of their gender and sex, but it does not explicitly protect intersex people.[29] There are also no laws that directly regulate the medical procedures and interventions that are often carried out on intersex people without their fully informed consent, such as non-consensual surgeries or hormone treatments. Because the law isn't specific, intersex people's bodily autonomy and rights are still being violated.

The Nigerian society currently experiences limited awareness and understanding of intersex issues, which makes it hard for intersex people to get along with others and avoid being discriminated against especially within healthcare settings where intersex bodies are constantly being pathologized. Because intersex Nigerians lack legal protections, their issues worsen, and they are more likely to have their human rights violated in ways such as being forced to have surgery, not receiving medical care, and being denied employment and education.

To improve the legal system, the Nigerian government should update existing laws and policies with comprehensive legislation that explicitly recognizes and protects the rights and experiences of intersex individuals. This legislation should address issues such as non-consensual medical interventions, discrimination, and the right to bodily autonomy. It should provide for visibility raising and public education campaigns, and also make sure that everyone has access to the right medical care, emotional and social support, and formal recognition of their gender identity and sex, no matter their sex characteristics.

[28] https://fida.org.ng/wp-content/uploads/2020/09/Violence-Against-Persons-Prohibition-Act-2015-1.pdf
[29] https://nigeriarights.gov.ng/files/constitution.pdf

SECTION 4 — Challenges in the Nigerian Healthcare System

Inclusive Healthcare Services

The healthcare system in Nigeria is not equipped to provide inclusive and affirming services for intersex individuals. In fact, Intersex individuals have reported experiences of healthcare providers insensitivity, disrespect and lack of provision of friendly and non discriminative services to their specific needs. Intersex-inclusive healthcare services are nearly nonexistent in Nigeria due to long-standing cultural and societal taboos around intersex persons and families. The lack of awareness and understanding about their health needs and obstacles leads to frequent misdiagnosis of intersex bodies, forced and unconsented "treatments," and a general disrespect for their physical and psychological well-being. This lack is matched with personal bias that also results in disbelief and insults, and the refusal to perform needed examinations, and examinations being carried out in violent ways or without the intersex person's consent. There are reported situations where an intersex person's genitals were on display during a medical examination and the doctor walked out only to bring in his medical students (without consent) to use the intersex body for educational practicals, they explained that their genitals were constantly being tossed without their consent and insensitive comments were made by the students while this was happening.

Intersex people for years have experienced stigmatization, discrimination and retraumatizing experiences with healthcare providers while accessing healthcare services within the healthcare system. The prevailing societal prejudices and misconception around intersex people are often subjected to judgement, ridicule, and mistreatment by healthcare providers. Consistently been abused, mistreated and called derogatory names, with some making very insensitive comments to outright denial of services or refusal to accommodate their medical needs. In other cases, the binary idea of the existence of either male or female bodies forms a perception that leaves healthcare providers with surprise when they see an intersex person at the facilities seeking healthcare and instead of honestly communicating about how unsure they feel about how to accommodate their specific needs, they rather become resentful and hostile.

One thing that has exacerbated the low awareness and understanding among the healthcare professionals and infiltrated the healthcare system is the absence of specific laws, guidelines or protocols. In Nigeria, there are no standard procedures or policies put in place by the Nigerian Medical Association or the government to ensure intersex people receive inclusive, diverse, appropriate and dignified services and affirming care. This is one thing that fuels the discrimination, nonchalance and condescending attitudes and treatments faced by intersex people and their families while attempting to or accessing healthcare services in Nigeria.

For example, a participant mentioned during our focus group discussion that they visited a private hospital in Abuja for an endometriosis condition that requires their primary hormones to be suppressed but according to the doctors, because of their intersex traits, they want to increase the hormones that are affecting their body instead because they are concerned about feminizing them and their organs because they were assigned female at birth and their gender identity isn't conforming to that. One of the doctors prescribed medication for them without informing them about the side effects but rather hastily coerced them into taking the medication so it could take effect immediately. Later,they googled about the medication when they got home to find out that one of the medication's side effects is supposed to make them grow larger breasts, which wasn't what they wanted.

This goes further to say that for health issues which are not related to their variation in sex characteristics, intersex people face challenges which the endosex (non intersex) population does not face in the healthcare facilities while accessing care. A 2018 study from the UK found that intersex respondents were more likely than non-intersex respondents to say that their general practitioner was not supportive. Intersex respondents were also more likely than non-intersex respondents to say they were too worried, anxious or embarrassed to go to their general practitioner.[30]

Inclusive healthcare policies are crucial for ensuring the well-being and rights of all individuals. Intersex people face unique challenges when it comes to accessing healthcare in Nigeria. They experience discrimination and inadequate medical care all through their lives. It is important to ensure that healthcare policies and practices in Nigeria are inclusive and sensitive to the needs of intersex people. This requires creating intersex informed guidelines for healthcare providers on how to provide respectful and affirming care to intersex patients, ensuring access to appropriate medical intervention when needed, and protecting intersex individuals from harmful practices such as non-consensual surgeries or unnecessary medical interventions.

Moreover, there is a need for public education and awareness campaigns to dispel myths and misconceptions surrounding intersex realities in Nigeria. These campaigns should seek to promote a more compassionate and understanding society in which intersex people are respected and protected. It is highly imperative to engage stakeholders such as healthcare providers, policymakers, and civil society organizations to collaborate and develop comprehensive guidelines and policies that address these unique healthcare needs of intersex people.

[30] https://www.gov.uk/government/publications/variations-in-sex-characteristics-technical-paper

Mental Health and Wellbeing

People who are born intersex often experience a lot of psychosocial challenges while navigating their life and relationships with others within societies that prioritise the binary understanding of sex and gender. The psychological and social impact of being an intersex person in Nigeria is profound; it contributes to shaping one's self-perception, ability to form and maintain relationships, and general wellbeing. Intersex people in Nigeria constantly struggle with feelings of confusion, low body image, shame, and isolation as they struggle to understand their own bodies.

The constant exclusion of the intersex body within the Nigerian culture when it comes fitting into the binary notion of male and female bodies causes a sense of being different, or abnormal, which often brings about feelings of negative self-image and low self-esteem amongst intersex people. Many intersex people have reported experiencing anxiety and depression as a result of being subjected to harmful medical interventions such as unnecessary surgeries without their personal, prior, and fully informed consent.



An intersex person shared a personal experience during the FGD, and we will call her Grace.

"My name is Grace, and I am a 26-year-old intersex person who lives in Enugu State. I honestly do not go to the hospital anymore because of the experiences I have had. In the past 12 years, anytime I visit the hospital, I always regret why I visited and why my mom always took me there. The last experience that made me decide not to go again until I had made enough money to afford a private hospital was when I told the doctor that the surgery I had when I was 16 years old had swelling around it, and I wanted him to look at it and tell me what the situation was. After he examined me and asked me some questions that I actually thought were irrelevant to the situation, he told me he was going to give me something for the swelling to reduce and then prescribe some medications as well. He injected a substance on me that made me wake two days later to discover that I was operated upon without my consent. He had been the doctor who carried out the surgery on me years ago and was trying to correct whatever mistake he made there with the approval of my brother without my consent. After some days, I was discharged, and I went home feeling like I had no control or power over my life and body, constantly questioning God about why I was created this way and for how long I would keep being used by these doctors for their failed attempts at correcting my body. For three months, I struggled with depression and stayed indoors. After that, I left home and haven't returned in three years"

Another common practice that has harmed the mental health of intersex people and the wellbeing of their families is medical practitioners withholding information about their patients' bodies. Concealing information from intersex people (including youth) delays the process of self-acceptance and increases shame and stigma. When an intersex person eventually learns the truth, they always ask: "Why did they hide this information from me unless there was something shameful about my body?" Secrecy also perpetuates the myth that variations in sex characteristics are extremely rare and taboo. In contrast, sharing information in an age-appropriate manner allows people to process it, influences self-empowerment, and promotes access to peer support.

Socially, intersex individuals face a lot of stigma and discrimination, which happens in schools, their homes, places of worship, and from their peers. There is a huge misunderstanding and misinterpretation of intersex variations in Nigeria, which exacerbates issues of social exclusion, bullying, and a lack of acceptance from family, friends, and society at large. Over time, these experiences have impacted the mental health and psychosocial wellbeing of intersex individuals by causing them emotional distress and a sense of not belonging, which makes them isolate themselves and suppress their true selves and abilities.

When it comes to relationships and disclosure about one's intersex body, intersex people worry and are unsure of how and when to tell their partner. This uncertainty stems from societal norms, personal insecurities, the fear of being judged, rejected, or discriminated against, and the possibility that the information will be used against them, leaving them feeling isolated and torn between internalized shame and self-protection. The psychological and emotional consequences of withholding or disclosing this information have a significant impact on their development and mental health. Many intersex people have had their sense of self-worth and acceptance shattered after being rejected by potential or actual partners due to variation in sex characteristics. The fear of rejection is what constantly leaves them with feelings of shame, self-doubt, and an ongoing need for reassurance, which they rarely receive in this part of the world.

For people who face these unique struggles and live in a society like Nigeria where the discourse on sex variations and gender is often stigmatized and misunderstood, there is still limited accessibility to inclusive and specific mental health support and services.. The lack of awareness and understanding of the specific intersex-affirming mental health needs and issues of intersex persons within the public and by healthcare professionals contributes greatly to this limited accessibility. Then there are religious and cultural biases, which create sociocultural stigma. Many healthcare providers, members of government institutions, and others continue to regard intersex people as taboo, associating them with witchcraft or divine purification, which manifests itself in their attitudes toward them and the reason for the nonrecognition of the mental health struggles and challenges of intersex persons and their families. In an interview from Intersex Nigeria study Beyond the Binary, a medical practitioner said that she believes intersex people were a mistake made by God, which is why God had given them the knowledge and abilities to fix their bodies.[31]

Limited understanding of intersex variations leads to misinformation, prejudice, and discrimination, which has an impact on the provision of inclusive and specific mental health services for intersex people. This is also why, tragically, in Nigeria, government institutions, health care providers, and CSOs make limited efforts to address the mental health concerns and needs of intersex people.

[31] https://intersexnigeria.org/resources/epublications

Ethics and Consent

Medical ethics is a very important component of the healthcare system in Nigeria and globally. It is the guiding principle in ensuring the delivery of quality, respectful and inclusive healthcare to all patients. However, when it comes to intersex individuals, these principles are often disregarded, leading to their exclusion in healthcare settings.

All over West Africa, intersex people face pathologisation and extreme violations of their human rights, because being intersex is seen, in itself, as a disorder. Many healthcare professionals approach intersex variations as a problem to be "fixed" or "normalised", rather than acknowledging the diversity of the human bodies as a natural occurrence. Medical professionals are very quick to offer medical intervention to intersex infants when their intersex traits are detected at birth without their informed consent or carrying out proper medical consultations. This is a common practice amongst healthcare providers in Nigeria which disregards a person's right to self determination and leaves lifelong repercussions on their health, wellbeing, identity and bodily autonomy.

Many intersex individuals in Nigeria undergo medical interventions without adequate information or choice. Since existence, intersex people have struggled with accessing affirming healthcare services and support in the most appropriate and friendly manner, which is almost non-existent in their world. They are constantly being forced to conform to irreversible medically unnecessary surgeries without their personal prior, free and fully informed consent.

Informed consent is the legal embodiment of the concept that each individual has the right to make decisions affecting his or her health. It is generally accepted that patients should consider the potential risks and benefits flowing from their medical decisions. A patient's understanding of the potential risks of proposed treatment is critical to medical informed consent;

"Personal" emphasises that only the intersex individual themself can consent to such an intervention and that parent or caretaker or medical professional cannot substitute for the intersex person's consent.

"Prior" refers to the timing of the consent, such that specific consent must take place before the intervention for which it is sought. For example, there is a common experience such that although parental or individual consent was given for a specific intervention, additional surgeries or interventions were simultaneously performed without consent, then followed by an attempt to gather consent after the fact for the additional interventions.

"Free" refers to the impact of power dynamics that may diminish a person's autonomy and pressure that may impact the individual's ability to consent. For example, reportedly pressure from healthcare providers has led intersex adults to consent to an intervention that they did not want just to finally have that pressure cease.

However, due to the lack of understanding and stigmatization surrounding intersex, many healthcare providers lack training and the necessary knowledge to address these issues adequately. Most of the public, including healthcare professionals, have limited awareness, and understanding of intersex variations. This lack of knowledge perpetuates harmful stereotypes, misconceptions, and discriminatory attitudes towards intersex individuals. They face social exclusion, prejudice, and even violence, which further restricts their ability to access sexual and reproductive healthcare services without fear of discrimination or judgment.

In the *Beyond the Binary,*[32] report published by Intersex Nigeria, there is a severe lack of knowledge about intersex people, the human rights violations they face and the specific needs that follow from these experiences exists among medical practitioners. This lack of awareness is matched with personal bias on behalf of healthcare providers that can result in disbelief and insults, the refusal to perform needed examinations, and examinations being carried out in violent ways or without full informed consent of the intersex individual. In a Focus Group Discussion (FGD) conducted by Intersex Nigeria in effort to ensure intersex voices and inputs for this toolkit, two intersex people spoke about the risk of sexual harassment in medical settings. Sharing their experiences; they mentioned how doctors touched their genitals as part of medical examination needed to understand the size and length of their genitals and often asked questions on how they have sex and need to describe it, if they do. They described it as an awkward and uncomfortable scenario they wish not to experience again. They mentioned that the genital examinations were unrelated to the need of their hospital visitation at that moment, and felt more as curiosity of the appearance of their genitals.

The invisibility of people born with innate variation in sex characteristics in society is another serious problem. Their life experience is often shrouded in secrecy and shame, and frequently, intersex people themselves are unaware of the surgeries or treatments that were performed on them during childhood. Access to medical records is often rendered very difficult, as is access to personal history, including childhood pictures and other memories. Intersex individuals who discover later in life may experience the same invasive treatment without their personal, prior, free and fully informed consent as intersex individuals who are identified during childhood. Even still, some doctors claim parental consent is enough to change the appearance or function of an intersex infant's genitalia, including cosmetic alterations to the clitoris just like in female genital mutilation (FGM).

Intersex genital "normalization" surgeries on the clitoris closely mirror female genital mutilation (FGM): they are typically non-therapeutic, driven by social norms about how bodies "should" look or function, not by medical necessity. Like FGM, these procedures raise grave human-rights concerns, violating bodily integrity, informed consent, and freedom from degrading treatment, yet the experiences of intersex children remain largely invisible due to medical secrecy, pathologizing language, and poor data. Survivors report lasting physical and psychological harm, while families often face pressure to consent without balanced information or alternatives. Greater recognition, transparent reporting, legal safeguards, and access to rights-based, deferrable care are essential to prevent these abuses.

[32] https://intersexnigeria.org/resources/epublications

Key Principles of Inclusive Policy Making

In the development of intersex inclusive healthcare policies, there are many key principles that are integral to promoting equal access and quality care for every intersex person. These key principles are non-discrimination, equity, and meaningful participation. These principles will guide policymakers in shaping policies that address the unique needs and realities of intersex people and their community, promoting a healthcare system that is fair, impartial and inclusive for every intersex person from birth to adulthood.

Non-discrimination principle: It is important to develop policies that explicitly state non-discrimination as a fundamental principle. This principle is rooted in the idea that everyone regardless of their Sexual Orientation, Gender Identity, Expression and Sex Characteristics (SOGIESC) identity should be treated equally and without prejudice. This means ensuring that individuals with intersex variations are not subjected to any form of discrimination or stigmatization when accessing healthcare services, and that these healthcare policies prohibit discriminatory practices based on factors such as race, religion, gender, sexual orientation, disability, or socioeconomic status. To accomplish this, healthcare providers must be trained on intersex issues and educated on the importance of providing inclusive and respectful care to intersex people, so that we can eliminate discriminatory practices and ensure intersex people's right to access friendly comprehensive healthcare services without encountering unjust barriers and biases.

Equity: This principle recognizes that everyone has unique healthcare needs and varying levels of resources to meet those needs. Healthcare policies should emphasize the importance of providing equitable access to healthcare for intersex people, meeting them at their point of need. This includes ensuring that all necessary healthcare services, including hormonal treatments, surgeries, and psychological support, are available to those who require them. Equity focuses on providing a fair distribution of healthcare resources and services, ensuring that marginalized or disadvantaged people receive the assistance they need to overcome barriers to healthcare access.

Participation: This principle recognizes that those directly affected by healthcare policies should have the opportunity to express their views, contribute to policy development, and influence outcomes. Engaging intersex individuals and their communities in the policy development process is important. It is crucial to establish mechanisms for meaningful participation and consultation, such as forming advisory committees or working groups consisting of individuals with lived experiences of intersex variations. Their insights and perspectives will inform the development of policies that are responsive to their individual needs and concerns.

Stakeholder Engagement

When developing intersex inclusive policies, it is critical to ensure that the voices and perspectives of intersex people are reflected in policy decision-making processes. To develop policies that are non-discriminatory, equitable, fair, and effective, intersex people, healthcare providers, and advocacy groups must all be involved in the policymaking process. This involvement not only enhances the legitimacy of the policies, but also offers necessary insight into the unique challenges intersex people face in Nigeria.

Intersex people, healthcare providers, and intersex advocacy groups (CSOs) will all play important roles in the intersex inclusive policy-making process, particularly in terms of rights and inclusive healthcare practices. Their participation is essential for several reasons.

1. Involving intersex people is critical because they are directly affected by these policies and share their lived experiences. Policies that have a direct impact on intersex people, such as those governing healthcare access, medical interventions, and legal recognition, should be tailored to their needs and preferences. Including their perspectives helps ensure that these policies are designed to meet their specific needs, rights, and concerns. When intersex people are directly involved in policy decision making processes, policymakers gain a better understanding of their lived experiences, which leads to more inclusive intersex policies while also promoting respect for their bodily autonomy, dignity, and self-determination.

2. Healthcare providers are at the forefront of providing care to intersex people. They also need to be involved because, as professionals, they have valuable knowledge and insights into the medical and psychological needs of intersex people in Nigeria, as well as a thorough understanding of the medical and ethical implications of the country's policy gaps and biases. Involving them will ensure that the proposed measures are consistent with best medical practices and are grounded in evidence-based research. Furthermore, healthcare providers can shed more light on the health implications of harmful policies and practices in the country that limit access to and the provision of quality healthcare services for intersex people in Nigeria. To avoid loopholes, only healthcare providers with extensive knowledge of intersex health and issues, as well as those without biases or personal conflicts, should be considered.

3. The inclusion of intersex-led and focused advocacy groups is critical for amplifying intersex voices and communicating their concerns to policymakers. Activists and advocacy groups have long worked tirelessly to promote intersex rights, challenge harmful societal norms, and advocate for legislative reforms. Their involvement will ensure that grassroots initiatives and community perspectives inform the policy-making process. They will also provide valuable input on policy drafts, offer alternative solutions, and help policymakers navigate the sensitive issues surrounding intersex rights, as well as provide documentation of the healthcare exclusions and discriminatory experiences that intersex people have while seeking and accessing healthcare services in Nigerian healthcare facilities.

Intersex Policy Design and Implementation

The healthcare needs of intersex people are mostly overlooked and largely misunderstood, which leads to discriminatory practices and inadequate support particularly in healthcare centers. Designing and implementing intersex inclusive and affirming healthcare policies is vital to ensure equal access to quality healthcare. Below are the key steps we should ensure to tick in designing and implementing these recommended policies.

Step 1: Understand and Acknowledge Intersex Realities.

The first step one needs to undertake in inclusive policy design for intersex people is to develop a comprehensive understanding of the various intersex variations that exist globally, their medical implications, and their specific healthcare needs. This will involve ensuring that intersex-led and focused advocacy groups, intersex activists, medical professionals, and affected individuals themselves are explicitly engaged. Also, building a robust knowledge management base, data, and resources on intersex issues will help form a strong foundation for an effective policy design.

Step 2: Raise Awareness and Educate

Raising awareness about intersex differences and their accompanying healthcare needs is critical for challenging cultural biases, social norms, religious beliefs and misconceptions. This step consists of the creation of educational programmes like Information, Education and Communication(IEC) materials, training courses, and other knowledge creation resource materials for healthcare providers, policymakers, human rights advocates, and the public. These initiatives should prioritize informed consent, bodily autonomy, and respect for the rights of intersex people, which are some of the basic needs of intersex people in Nigeria.

Step 3: Policy Development and Implementation

After laying the groundwork for understanding intersex realities and raising awareness about their healthcare needs and policy gaps, the policy development phase will begin, with a focus on translating knowledge and understanding into actionable policies. This is the point where, intersex individuals, healthcare providers, intersex activists, intersex-led and focused advocacy groups and policy makers will sit together. This phase includes drafting of guidelines, protocols, and standards of care that healthcare providers in Nigeria will follow when engaging and providing care to intersex patients. These policies will address issues such as comprehensive healthcare coverage, appropriate medical interventions, fully informed consent, confidentiality, access to medical reports, mental health support, and the prohibition of unnecessary or coerced surgeries amongst many others.

Step 4: Monitoring and Evaluation

At this point, it is critical to establish mechanisms for monitoring and evaluating the effectiveness of the developed intersex-inclusive healthcare policies. Data collection on healthcare outcomes, patient experiences, and barriers to access care should be a part of routine practice. Regular evaluation of policies will help identify gaps, challenges, and areas for improvement.

Step 5: Collaboration and Engagement

Collaboration with intersex advocacy organizations, healthcare professionals, policymakers, and affected individuals is critical throughout the policy development and implementation process. Engaging intersex individuals in decision-making processes ensures their voices and experiences are included and respected.

Examples of Intersex Policy Best practices

Kenya has taken significant steps toward recognising intersex persons within legal and civil registration systems. Court rulings and administrative reforms have improved the recognition of intersex individuals in official documents and strengthened public awareness of intersex rights. These efforts illustrate how legal recognition can reduce invisibility and support access to services, including healthcare.

The Australian Capital Territory established a regulatory framework that requires approval processes for certain medical treatments affecting variations in sex characteristics. The policy emphasises delaying deferrable procedures until individuals can meaningfully participate in decision-making. This approach demonstrates how healthcare regulation can balance medical considerations with human rights protections.

In Malta, the Ministry of Education and the Ministry of Health collaborated to introduce intersex-inclusive comprehensive sexuality education programs in schools. These programs educate students about intersex issues, promoting understanding and acceptance from an early age.

Germany, through its Network for Intersex Discourses, developed the "Interdisciplinary Consensus Statement on the Care of People with Variations of Sex Development." This consensus statement provides guidance for healthcare providers on the ethical and evidence-based management of intersex conditions, promoting more inclusive and supportive healthcare practices.

In Argentina, the National Institute against Discrimination, Xenophobia, and Racism (INADI) works closely with the community to collect data and monitor the implementation of intersex inclusive policies. This data-driven approach provides valuable insights for policy refinement.

The Netherlands Institute for Human Rights actively involves intersex individuals in policy discussions through its Intersex Expert Group. This approach ensures that policies are grounded in the lived experiences of intersex people and their specific healthcare needs.





Guidelines for Healthcare Providers

- Respect the human rights and identities of infants, children, adolescents, and adults with intersex variations. This includes delaying non urgent and irreversible medical interventions like genital surgeries.

- Ensure full informed consent is gained from the patient for any medical or surgical treatment that may impact genital appearance, gonads or fertility, and for any treatment that appears to have a rationale in social, cultural or gender stereotypes, such as the idea that boys need to be able to stand up to urinate.

- Health service providers should ensure that all procedures, treatments, and surgeries are explained clearly and comprehensively to intersex individuals and their families. Obtain informed consent, providing them with all relevant information, potential risks, and alternatives, and allowing them to make well-informed decisions about their own healthcare without any form of coercion or influence.

- Healthcare providers should ensure that intersex people have access to expert and individually tailored psychological and psycho-social counselling and support for all concerned individuals and their families, from the time of self-referral or diagnosis for as long as necessary.

- Healthcare professionals and directors should regularly evaluate, update healthcare policies and practices to ensure that they align with current best practices around the world and are of international human rights standards. They should regularly seek feedback from intersex individuals and other marginalized groups to involve them in decision-making processes, ensuring that their voices are heard, valued, and considered when improving healthcare services.

- Use intersex inclusive language, it provides helpful guidelines on how to speak with and about intersex people in a respectful manner.

- Healthcare providers should tailor healthcare plans in a way that it addresses the specific needs and goals of each intersex patient. Inclusion and the recognition that there is a wide range of intersex variations, and that each person's experience may be different.



Recommendations for Government Agencies

- The Federal Ministry of Justice (FMOJ), the Federal Ministry of Health and Social Welfare (FMOHSW), intersex-led organisations, and other CSOs should form collaborative efforts to ensure the formulation, enactment, implementation, and enforcement of national laws and policies to end medically unnecessary "normalising" treatment of intersex persons, including irreversible genital surgery and sterilisation, when it is enforced or administered without the free and fully informed consent of the person concerned. Sex assignment treatment should be available to intersex individuals at an age when they can express their free and fully informed consent. We must respect the right of intersex persons not to undergo sex assignment treatment.

- Government institutions like the Federal Ministry of Health and Social Welfare (FMOHSW) should establish obligatory training for medical professionals, such as doctors, midwives, psychologists, and other professionals working in the health sector (e.g., reception desk staff), in order to ensure that intersex individuals and their families have access to adequate healthcare and are protected from discrimination. The training should include information about the vulnerability and special needs of intersex people in medical settings.

- The National Human Rights Commission in Nigeria should be active in outreach and public support for intersex people, including children. They should clearly declare their mandate to work on issues related to intersex people and provide victim support services to them in cases of violations.

- The National Identity Management Commission (NIMC) should ensure flexible procedures are observed in assigning and reassigning sex/gender in official documents.

- The Federal Ministry of Health and Social Welfare (FMOHSW) and the Department of Planning, Research, and Statistics (PRS) in collaboration with intersex advocacy groups should carry out research into the situation and human rights protection needs of intersex people in different settings. There is an urgent need to improve public awareness and professional training about the problems encountered by intersex people and in the development of measures that enhance the full enjoyment of their human rights.

Recommendations for Government Agencies

- The Federal Ministry of Health and Social Welfare (FMOHSW) should ensure to create Standard Operating Procedures for intersex births in hospitals that can include; intersex register and record in public and private hospitals, including teaching hospitals, and births facilitated by traditional birth attendants to ensure clear documentation of intersex births across the country.

- The Federal Ministry of Health and Social Welfare (FMOHSW) should make it mandatory for intersex education to be included in the medical curriculum in medical school for all undergraduate training.

- The Federal Ministry of Education (FMOE) should consider including intersex people and the existence of variation in sex characteristics in a positive and empowering way in school curricula, text books, and education materials in order to increase the understanding of the general public on the quality of sex as a spectrum and the existence of intersex people and to improve the self-confidence of intersex children and adolescents.

- Other government agencies, including the Federal Ministry of Health and Social Welfare (FMOHSW), should invest in funding intersex groups and organisations so that they can engage in projects offering training for medical professionals, midwives, psychologists, and other professionals working in the field of physical and mental health.





Call to Action for CSOs

Intersex advocates are committed to ending these harmful practices and allowing intersex people to make choices about their own bodies.

- Child rights and human rights organisations should organise human rights-based trainings about the existence of intersex people and about the specific needs of intersex people for other human rights CSOs to strengthen their capacity around understanding these needs, including them in their own health advocacy asks, during training of healthcare providers, and to help them make the right referrals when they come in contact with intersex individuals with these needs.

- Intersex Awareness Day is observed every October 26 and marks the anniversary of the first public protest against non-consensual infant genital surgeries in the United States in 1996. Learn about the history of intersex advocacy and what you can do to uplift intersex voices.

- CSOs can support the efforts of Intersex-led and focused organizations to document and disseminate good practice in advocacy work on Intersex inclusion in the healthcare system and amongst healthcare providers in Nigeria.

- CSOs should collaborate with intersex-led and focused organizations to ensure adequate participation and inclusion of intersex agenda in health sector reviews and other relevant processes that can promote intersex inclusion in healthcare.

- Familiarise yourself and your organisation with international declarations, documents, treaties and procedures that promote intersex rights including inclusive healthcare access and understand the national health context, so that you appear knowledgeable, professional and responsive to the many pressures from the government and effectively advocate for intersex health inclusion.
- CSOs working on female genital mutilation(FGM), children and adolescents should develop webinars, e-learning materials and create an online space for CSOs working on intersex inclusion in healthcare systems to share experiences and case studies and allow collaborative exchange and communications

- CSOs who have longer years of experience and representatives within the policy sectors should provide support for Intersex-led and focused organizations to leverage on that established partnership to disseminate their reports at national, regional and global levels to reach the targeted audience and for faster outcomes.

Conclusion

Intersex individuals and advocates across the globe began challenging current harmful medical practices on intersex bodies in the mid 1990s. In a relatively short time, the intersex movement has increased public awareness about health care issues facing the intersex community and commenced a productive dialogue and interventions with healthcare professionals. However, approximately 95% of medical practitioners, still recommend that parents of intersex newborns consent to medically unnecessary harmful genital surgery on behalf of their children.[33] Most parents follow their doctor's advice and approve of these procedures.

Although the Child Rights Act (CRA) of Nigeria[34] grants great deference to parents regarding the medical decisions made on behalf of their children, traditional informed consent procedures are inadequate to protect the best interests of Intersex children in Nigeria. Policies and procedure should be put in place to control these processes for the following reasons;

- **The surgeries negatively affect important rights.** These surgeries have the potential to affect a person's life abilities, create life time complications and pain, scar, trauma and low body esteem, etc. Surgeries that affect an important constitutionally protected right should only be performed with the informed consent of the person undergoing the procedure. If the person being subjected to the treatment is unable to consent, additional safeguarding measures should be imposed to ensure that these important rights are not violated.

- **Parents may not be able to completely understand their child's best interests.** When parents make these decisions, they are visualizing their infants as children and not as adult sexual beings. One study[35] showed that parents consent to these medical interventions because they are concerned about the emotional well-being of their children. Other studies[36,37] indicated that the majority of adults would not want their parents to consent to these surgeries, especially if there is a risk that the medical procedure will affect their ability to engage in satisfactory sexual relations. Therefore, these procedures should not be performed under traditional parental consent practices

[33] Fixed at birth: Medical and legal erasures of intersex variations. A Kennedy. University of New South Wales Law Journal, The 39 (2), 813-842, 2016. 24, 2016.
[34] Child's Rights Act, 2003, Act No. 26, Laws of the Federation of Nigeria, 2003.
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- **These medical procedures are based on sex, gender, and sexual orientation stereotypes.** Although some infants with atypical genitalia must undergo interventions to repair conditions that threaten their health, most intersex bodies do not impose any health risks. Most of these surgeries are undertaken for purely cosmetic reasons. These surgeries are performed based on false sex, gender, and sexual orientation stereotypes, including the assumptions that women prioritize cosmetic appearance over sexual pleasure, men need a penis that will allow them to urinate in a standing position, and all men and women will want to engage in traditional intercourse. Sex stereotypes should not be used to support life-altering health care practices.

- **These surgeries are undertaken not because these infants are disabled but based on the perception that they are impaired and need to be "fixed".** Most infants with intersex variations are able to participate in all major life functions and are completely healthy and not disabled or impaired. Unfortunately, Intersex babies are subjected to invasive, irreversible, and potentially harmful medical interventions to bring their bodies into conformity with societal norms. Often the result of these unnecessary medical interventions cause physical and emotional problems that previously didn't exist such as sterility, anxiety and depression. Furthermore, People who are perceived as disabled should not be subjected to invasive procedures to conform their bodies to societal expectations. The cure is not the surgical alteration of the child; the cure is educating society to accept bodies that are diverse.

- **No study has proven that cosmetic genital surgeries benefit intersex people, while evidence exists that these procedures can lead to serious physical and emotional harm.** Given the interests at stake, courts and legislatures should consider imposing legal safeguards to ensure that health care decisions made on behalf of intersex children are actually in their best interests. Legal institutions should consider imposing a moratorium on these procedures until they have been proven to be beneficial, enhancing informed consent practices for these procedures, or requiring external oversight or approval by a court or hospital ethics committee. Feminists, Intersex rights activits and human rights advocates have long spoken up on how discrimination based on sex, gender, and sexual orientation stereotypes constitutes unlawful sex discrimination.

- **Discrimination in health care should be prohibited.** The Federal Ministry of Health and Social Welfare (FMOHSW) should develop ethical procedures and medical protocols that safeguard the lives of intersex persons, including ensuring that any unnecessary surgery, including forced sterilization, or the assignment of gender only takes place when the child is able to participate in decision-making, in the best interests of the child. The principle of participation and involvement extends to all intersex persons. The meaningful involvement of intersex persons should always form a central part of any medical decision.

- **The involvement of intersex persons in developing health policies and guidelines is equally critical, to protect and promote human rights.** In addition, despite the rise in the dissemination of information on intersex persons, through social media, intersex people in Nigeria continue to experience a lack of understanding of their status beyond a binary understanding of male or female bodies. This leads to the creation of stereotypes, resulting in marginalization and discrimination within healthcare spaces. Increased efforts to ensure the participation of intersex persons in decision making forums and their representation in healthcare structures will help to strengthen their voice.

- **There are various opportunities in Nigeria for further protecting and promoting the rights of intersex persons,** based on law and policy reform, guided by global, regional and national human rights principles and lessons learned from comparative national legislation in various jurisdictions such as Kenya, South Africa and Malta, which show the importance of, among other things, clearly defining and providing for the rights of intersex persons in both broad and specific laws and policies across various sectors particularly healthcare; prohibiting unnecessary medical interventions; allowing for changes to critical identity documents; increasing awareness of the rights of intersex persons; and strengthening access to justice for rights violations.

We, the intersex persons, demand; recognition of our experiences because our variations are part of human diversity. When we seek healthcare services, please accord us privacy because we are not an experiment; being seen by a team of doctors when it is not necessary in one sitting is dehumanizing. It is important that health personnel provide us with unbiased healthcare support and government agencies are equipped with adequate and comprehensive information through policies and laws to support intersex persons and their parents, with accompanying psychosocial support for our wellbeing and development.

Appendices

Glossary of Terms

CSO	Civil Society Organisation
IGM	Intersex Genital Mutilation
FGM	Female Genital Mutilation
IEC	Information, Education And Communication (Materials)
FMOH	Federal Ministry Of Health
FMOJ	Federal Ministry Of Justice
FMOE	Federal Ministry Of Education
FGD	Focus Group Discussion
VAPP	Violence Against Persons Prohibition Act
CRA	Child Rights Act
DPRS	Department Of Planning, Research, And Statistics
NIMC	The National Identity Management Commission
NHC	National Human Rights Commission
NGO	Non-Governmental Organisation
SGBV	Sexual And Gender-Based Violence
SRHR	Sexual And Reproductive Health And Rights
UN	United Nations
WHO	World Health Organisation

Additional Resources

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- Spain – Law 4/2023 (Art. 19). Regulates intersex health; bans genital modification practices on children under 12 (health exceptions), sets consent rules for 12–16. Juridicas NewsILGA Database
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Americas

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Integrating Intersex Perspectives into Healthcare Policies for Inclusive Practice in Nigeria is a groundbreaking policy and practice toolkit designed to transform healthcare systems through dignity, autonomy, and human rights. Drawing from lived experiences, international human rights standards, and years of advocacy, this publication provides practical guidance for healthcare providers, policymakers, and civil society organisations seeking to build inclusive and affirming healthcare environments for intersex people in Nigeria.

Through evidence-based analysis, real community voices, and actionable recommendations, the toolkit challenges harmful medical practices and advances a vision of healthcare that respects bodily autonomy and diversity in sex characteristics. It stands as both a resource for institutional reform and a call to action toward a more just and equitable healthcare system in Nigeria.



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